

LECTURE 1 — AMPUTATION (Revision Table)

Bold = high-yield.

INDICATIONS & VARIETIES	
Topic	Key points
Indications (3 D's)	Dead/dying (PVD, trauma, burns, frostbite); Dangerous (malignancy, lethal sepsis, crush syndrome); Damn nuisance (pain, malformation, recurrent sepsis, loss of function)
Provisional	Healing unlikely → amputate distal, loose flaps over pack, re-amputate later
Definitive end-bearing	Weight through end; scar NOT terminal ; bone solid (near joint)
Definitive non-end-bearing	Commonest ; scar can be terminal ; all UL + most LL (through-knee, Syme's)
Site of election	Prosthetic design + below most distal palpable pulse . Too short → slips; too long → ulcerates

TECHNIQUE & COMPLICATIONS	
Topic	Key points
Technique	Tourniquet (unless ischaemia); skin flaps = 1.5× limb width (long posterior flap for BK); myoplasty ; nerve divided proximal; fibula 3cm shorter than tibia ; suction drain
Levels	Transfemoral ≥12cm; transtibial 14cm; Syme's (above malleoli) , Chopart (midtarsal) , Lisfranc (TMT)
Early complications	Secondary haemorrhage, flap breakdown (ischaemia/tension), gas gangrene
Late complications	Skin (ulcer, SCC , verrucous hyperplasia); painful neuroma (Tinel's sign, NOT tumour) ; phantom limb (hard to treat); joint stiffness/deformity; bone spur/fracture
Prosthesis	Comfortable, functional, presentable, early fit; LL weight via greater trochanter/patellar tendon/upper tibia

LECTURE 2 — MSK INFECTION (Revision Table)

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SEPTIC ARTHRITIS & OSTEOMYELITIS		
Aspect	Septic arthritis	Osteomyelitis
Definition	Joint infection by pus organism	Bone marrow infection (acute/chronic)
Organism	Staph aureus ; H. influenzae (1–4yr) ; strep, gonococcus (adults)	Staph aureus >70% ; Salmonella in sickle cell ; H. influenzae (1–4yr)
Clinical	Neonate = septicaemia (exam umbilical + IV site); child = hip/knee/shoulder, pseudoparesis, held in ease ; adult = superficial joints	Mainly children ; progression: inflammation → suppuration → necrosis → new bone → chronicity
Investigations	↑WBC/ESR/CRP, joint aspiration (pus → C&S) , blood culture, US, X-ray	CBC/ESR/CRP, blood + pus culture, X-ray, US
Treatment	Antibiotics (flucloxacillin/cephalosporin), drainage if no response in 72h , splintage	Empirical antibiotics first , drainage if needed, splint/rest
Complications	OM, dislocation, epiphyseal damage (deformity/LLD) , ankylosis, lethal septicaemia	Altered growth, septic arthritis, pathological fracture, chronic OM , death

DIABETIC FOOT	
Aspect	Key points
Definition	Infection/ulcer below malleoli in long-standing DM. 30% neuropathy/angiopathy; 25% develop foot
PVD	Atherosclerosis below knee ; claudication, cold smooth skin, weak pulses; vascular ulcers painful + tender (toes, heel, 1st MT)
Sign	Rocker-bottom (Charcot) deformity = diagnostic + disastrous
Management	Multidisciplinary; prevention (foot care, diabetic clinic); glycaemic control + C&S antibiotics + saline dressing + debridement (biofilms) ; ray excision, Ilizarov, total contact casts